June 16, 2025

Mehmet Oz, M.D.
Administrator, Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
7500 Security Boulevard
Baltimore, MD 21244

RE: CMS Request for Information: Health Technology Ecosystem
File Code CMS-0042-NC/CMS-2025-0050-0031

Dear Administrator Oz,

I am writing to submit comments on behalf of Veradigm LLC to the Request for Information regarding the health technology ecosystem. We appreciate the opportunity to contribute our insights to help improve the healthcare experience for Medicare beneficiaries and drive development and adoption of digital health solutions that empower patients and improve care.

With a robust platform offering clinical, business, and interoperability solutions for a diverse range of practice types—from solo practitioners and independent practices in rural America to large multi-specialty groups impacting patients all across the nation—Veradigm supports thousands of providers across various care settings. Our three decades of experience in partnering with this extensive network enables us to provide informed feedback on the proposed rule.

Specifically, we have the following comments:

Patients & Caregivers

PC-14 a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials.

A key challenge in encouraging patients and caregivers to adopt digital identity credentials is the added complexity and burden of managing yet another account. Users often prefer the convenience of biometric logins like FaceID or TouchID and may forget which third-party credentialing service they used, leading to access issues. Additionally, support teams are limited in their ability to assist with

third-party account recovery, and there is a common misconception among patients that their clinical data is being shared with the authentication provider, which can further deter adoption.

PC-14 b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

From our perspective, there is little perceived benefit to patients or caregivers in the broader adoption of digital identity credentials. Users are not requesting this functionality, and it is often viewed as an additional requirement that adds complexity without delivering clear or tangible value.

PC-14 c. What are the potential downsides?

The use of digital identity credentials introduces additional friction for users, including the need to create and manage another account, remember separate credentials, and recall which sign-in method was used. These extra steps can negatively impact user experience, especially since login is a frequent and essential interaction. Furthermore, implementing such systems would require development resources and could introduce additional costs for both the organization and potentially the patient, diverting focus from other enhancements that users are actively requesting.

PC-14 d. How would encouraging the use of CSPs improve access to health information?

We do not believe that encouraging the use of third-party credential service providers would meaningfully improve access to health information. Patients overwhelmingly prefer the simplicity and immediacy of using their mobile devices—such as FaceID or TouchID—as the key to their data, rather than navigating additional identity verification layers.

PC-14 e. What role should CMS/payers, providers, and app developers have in driving adoption?

We do not plan to implement support for third-party digital identity credentials unless it becomes a regulatory requirement. At this time, we do not see a compelling reason to prioritize adoption, as it does not align with user demand.

Payers

PA-1 What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

While we are familiar with TEFCA, our organization is not directly involved in exchanging for Payment or Operations use cases today. Our TEFCA implementation is primarily focused on clinical data exchange, though future use cases do include options for payment and operations, such as electronic prior authorization and HEDIS measures. However, we have not observed significant adoption or impact of TEFCA in our client base, and its broader implementation—particularly beyond pilot

programs—appears limited. Currently, most payer data exchange is centered around supporting retrospective chart reviews. To better align with industry needs, we believe risk adjustment should be established as a mandatory response use case within TEFCA.

PA-2 How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

CMS can support acceleration by aligning regulatory requirements with the realities of current data exchange capabilities. For example, RADV compliance requirements are misaligned with the data typically available in CCDAs, which often lack physician notes necessary for retrospective risk adjustment—perpetuating reliance on legacy chart-pull methods. Additionally, many large payers have invested heavily in proprietary infrastructure over the past decade, making integration with TEFCA and FHIR-based APIs a significant technical lift. There is also widespread confusion among payers about endpoint management, especially for organizations with multiple lines of business. Developing a unified routing mechanism for APIs is seen as more complex than implementing FHIR transactions themselves. CMS could help by supporting centralized API directories, clarifying endpoint expectations, and recognizing the scale of transformation required to meet upcoming mandates like electronic prior authorization.

PA-3 How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

To encourage adoption of digital identity credentials such as Clear or ID.me, CMS should consider providing funding or incentives to offset the cost of implementation. Clear, consistent standards that apply universally across payers are essential to avoid fragmented, proprietary approaches that hinder interoperability. While there is broad support for advancing digital identity solutions, the scale of technical and operational change required is significant. Without a unified framework and clear guidance, each payer may interpret requirements differently, leading to inconsistent implementations that are difficult to scale. Mandates alone may not be sufficient—supporting infrastructure and realistic timelines are also critical to success.

PA-4 What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

A nationwide provider directory would offer significant value to payers—particularly if it is well-maintained and accurately reflects where providers are rendering care. Reliable access to FHIR endpoints and digital identity credentials could streamline interoperability and reduce administrative burden. However, many legacy payer systems still rely on provider address as the primary matching criterion due to outdated EHR infrastructure and inconsistent availability of more precise identifiers like NPI and TIN. Addressing these foundational data challenges is essential to realizing the full benefits of such a directory.

PA-5 What are ways payers can help with simplifying clinical quality data responsibilities of providers?

Payers can support providers by focusing on eliminating duplicative reporting requirements and streamlining clinical quality data submission processes. While these changes may be complex to implement, the long-term benefits in reducing administrative burden and improving data consistency would be substantial. Clear coordination and simplification efforts are essential to making quality reporting more manageable and scalable for providers.

PA-5 a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

There is strong interest among payers in EHR technology that supports bulk extraction of clinical quality data, particularly when the extracted data includes the specific elements required by their quality analytics engines. This approach could reduce the burden on provider-side technology and streamline quality measurement processes, provided the data is complete and standardized to meet payer needs.

PA-5 b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

Consolidation of interoperability and quality reporting responsibilities should begin with universal adoption of standardized data formats and reporting frameworks. This would allow investments in technology to serve multiple purposes across CMS and other payers. However, while real-time data extraction is often discussed in the context of digital quality measures, it can place significant strain on systems actively used for patient care. A more practical approach may involve scheduled, non-disruptive data pulls—such as nightly extractions—that balance timely access with system performance. Flexibility in timing and method of data exchange will be key to sustainable implementation.

PA-7 How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

Rather than focusing solely on encouraging complaint submissions, CMS should prioritize widespread education across the payer community on what constitutes information blocking. Many payers encounter challenges such as high-cost data access or special agreements imposed by large health systems, but there remains significant confusion around what qualifies as a reportable offense. Clearer guidance would help ensure that only meaningful, well-founded complaints are submitted—avoiding unnecessary strain on the system and promoting more constructive resolution paths. Encouraging collaboration and dialogue before escalation may also be more effective in advancing data exchange than formal complaints alone.

Technology Vendors

TD-1 What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

In the short term, CMS should prioritize the development and adoption of robust, actionable data standards that enable meaningful interoperability across systems. Current standards are too rudimentary to support valuable use cases, limiting developer engagement. In the long term, CMS should focus on expanding access to relevant datasets—such as cost performance data—and ensure that APIs are built to support real-world workflows. Stimulating developer interest also requires aligning incentives and infrastructure to support both EHR and non-EHR vendors in creating solutions that are practical, scalable, and beneficial to Medicare beneficiaries and their caregivers.

TD-2 a. What additional data would be most valuable if made available through CMS APIs?

Before expanding the volume of available data, efforts should focus on fully leveraging and standardizing the data already accessible. Additional data is only valuable if it is readily available, structured using consistent code sets, and supported by mature interoperability infrastructure. Enhancements such as dynamic patient lists and subscription-based data delivery would significantly improve efficiency and reduce system strain. Without these foundational capabilities, simply adding more data risks overwhelming systems and users without delivering meaningful value.

TD-2 d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

Before introducing new APIs, CMS and ONC must prioritize the development and adoption of robust, standardized, and coded data frameworks. Innovation and interoperability cannot thrive without mature standards in place. Releasing APIs ahead of finalized standards leads to confusion, inefficiencies, and frustration across the ecosystem. A standards-first approach is essential to ensure that any new APIs are meaningful, actionable, and capable of supporting real-world use cases for patients and providers alike.

TD-3 Regarding digital identity implementation:

We are supportive of advancing patient digital identity solutions, recognizing their potential to enhance secure access and interoperability. However, successful implementation will require a clear understanding of the requirements and a realistic development timeline. Health IT developers will need a minimum of 18 to 24 months from the final rule to fully implement digital identity functionality across patient-facing use cases. This timeline accounts for the complexity of integration, the need for robust standards, and the significant development effort required to ensure secure and scalable adoption.

TD-3 a. What are the challenges and benefits?

We are supportive of advancing patient digital identity solutions, recognizing their potential to enhance secure access and interoperability. However, successful implementation will require a clear understanding of the requirements and a realistic development timeline. Health IT developers will need a minimum of 18 to 24 months from the final rule to fully implement digital identity functionality across patient-facing use cases. This timeline accounts for the complexity of integration, the need for robust standards, and the significant development effort required to ensure secure and scalable adoption.

TD-3 b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3IAL2/AAL2 CSPs) impact cybersecurity and data exchange?

We are supportive of advancing patient digital identity solutions, recognizing their potential to enhance secure access and interoperability. However, successful implementation will require a clear understanding of the requirements and a realistic development timeline. Health IT developers will need a minimum of 18 to 24 months from the final rule to fully implement digital identity functionality across patient-facing use cases. This timeline accounts for the complexity of integration, the need for robust standards, and the significant development effort required to ensure secure and scalable adoption.

TD-3 c. What impact would the mandatory use of the OpenID Connect identity protocol have?

We are supportive of advancing patient digital identity solutions, recognizing their potential to enhance secure access and interoperability. However, successful implementation will require a clear understanding of the requirements and a realistic development timeline. Health IT developers will need a minimum of 18 to 24 months from the final rule to fully implement digital identity functionality across patient-facing use cases. This timeline accounts for the complexity of integration, the need for robust standards, and the significant development effort required to ensure secure and scalable adoption.

TD-4 How can CMS better encourage use of open, standards -based, publicly available APIs over proprietary APIs?

To promote the use of open, standards-based APIs, CMS should coordinate closely with ONC to ensure alignment with certification requirements and technical standards. Developers are often required to implement APIs before standards are fully mature, which leads to inconsistent adoption and reliance on proprietary solutions. Encouraging adoption through positive incentives—such as financial support or performance measures for providers using standards-based APIs—would help drive demand and justify the investment needed to expand FHIR-based capabilities. A standards-first, incentive-aligned approach is essential to achieving scalable, interoperable health IT infrastructure.

TD-5 How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

A centralized, nationwide directory of FHIR endpoints could improve access to health information by simplifying discovery and connectivity for patients, providers, and payers. However, such a directory should be managed by a federal entity like CMS or ONC, rather than individual vendors. While patient-facing endpoints are currently required and could be aggregated, provider-facing endpoints involve more complexity due to licensing, authorization, and contractual relationships. Therefore, broad publication of provider endpoints may not be appropriate at this time. Any implementation must account for these distinctions and ensure alignment with existing certification and access frameworks.

TD-6 What unique interoperability functions does TEFCA perform?

TEFCA distinguishes itself by offering a standardized, federally governed framework that supports a broader range of use cases than other networks—such as treatment, healthcare operations, public health, individual access services, and government benefits determination. Unlike other networks, TEFCA ensures all participants meet uniform technical and security requirements, creating a more consistent and trustworthy environment. Its upfront vetting process enhances confidence in data exchange partners, and its governance structure allows for future expansion. While these capabilities are well-documented, CMS and ONC should already be familiar with them, as they are foundational to TEFCA's current design and implementation.

TD-6 b. Are there redundant standards, protocols or channels or both that should be consolidated?

Given that TEFCA is still in early stages of adoption, it is premature to determine whether there are redundant standards or protocols that require consolidation. However, as private networks like Carequality and CommonWell evolve, some overlap may naturally resolve—especially as major participants like Epic transition to TEFCA. Rather than introducing new frameworks, CMS and ONC should focus on fully implementing and supporting TEFCA, ensuring it becomes the unified, standards-based foundation for nationwide interoperability. This includes incentivizing participation through mandatory requirements rather than optional measures, to ensure consistent adoption and return on the significant investments already made by stakeholders.

TD-7 To what degree has USCDI improved interoperability and exchange and what are its limitations?

USCDI Version 1 has been in practical use for approximately two years and has contributed positively to advancing interoperability by establishing a standardized set of data elements for exchange. Its implementation has helped align stakeholders around a common framework. However, its scope remains limited, and further expansion is needed to support more comprehensive and meaningful data sharing. Continued evolution of USCDI is essential to address emerging use cases and ensure that the data exchanged is both actionable and relevant across diverse healthcare settings.

TD-7 a. Does it contain the full extent of data elements you need?

The completeness of USCDI depends heavily on the specific use case. While USCDI Version 1 has provided a valuable foundation for interoperability, it lacks the breadth and depth required for more complex needs such as public health reporting, clinical quality measurement, and specialized domains like mental health or maternal care. The forthcoming USCDI+ extensions are critical to addressing these gaps. Continued expansion and refinement of USCDI—tailored to diverse use cases—will be essential to fully support meaningful data exchange across the healthcare ecosystem.

TD-7 c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

Expanding USCDI with additional data elements can add significant value, but only if done through a structured, standards-based approach. The introduction of USCDI+ domains—such as public health, mental health, and maternal health—is essential to meeting diverse use case needs. However, deviating from standardized data definitions risks confusion, inefficiencies, and reporting challenges. To avoid scoping issues, all expansions must be carefully governed and aligned with established interoperability standards.

TD-7 d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

Deviating from standardized data definitions risks confusion, inefficiencies, and reporting challenges. To avoid scoping issues, all expansions must be carefully governed and aligned with established interoperability standards.

TD-8 What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The evolution of ONC's certification criteria has been a necessary and deliberate process. Early efforts focused on foundational capabilities, while more recent shifts toward interoperability and API-enabled functionality—such as those proposed in the HTI-1 and HTI-2 rules—represent critical progress. These newer criteria are among the most effective, as they prioritize data exchange and real-world usability. However, it's important to recognize that implementing certification requirements based on immature standards can lead to unrealistic expectations and operational challenges. Continued alignment with maturing standards and a focus on practical, scalable implementation will be key to long-term success.

TD-9 a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

Prioritizing API-enabled capabilities—particularly FHIR-based APIs—over traditional software functionality offers significant benefits in terms of privacy, security, and data efficiency. Unlike broad document exchanges (e.g., CCDAs), APIs allow for more precise, purpose-driven data sharing, ensuring only the necessary information is transmitted. This reduces the risk of overexposure, aligns better with

patient consent, and minimizes the potential for misuse of excess data. Emphasizing API-based exchanges supports a more secure, scalable, and patient-centered approach to health information sharing.

We also recognize the importance of the forthcoming ONC final rule on the HTI-2 proposed rule, which is expected to expand certification requirements around APIs. However, API functionality is only as effective as the quality and efficiency of the data capture workflows within the software. To ensure APIs deliver meaningful value, developers must be given adequate time not only to build the technical components but also to design user-friendly, low-burden workflows that support accurate and complete data entry by clinicians.

TD-9 b. What would be the drawbacks?

While APIs are clearly the future of interoperability, their effectiveness depends on mature standards and robust underlying software functionality. APIs can only deliver value if the data they expose is accurately and efficiently captured through well-designed workflows. Shifting focus too heavily toward APIs without ensuring the software supports high-quality data entry and clinician usability risks undermining both data quality and user experience. Additionally, developers need adequate time not only to build the technical components but also to design workflows that minimize burden and preserve patient privacy. Standards must guide what is shared—developers cannot be expected to parse unstructured data blobs to determine relevance.

TD-10 For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

Our team recognizes the intent behind the 21st Century Cures Act to promote seamless access, exchange, and use of electronic health information. However, we emphasize that achieving this goal "without special effort" is currently unrealistic without first ensuring robust, universally adopted data standards. The term "all data elements" is overly broad and technically challenging given the evolving nature of standards like FHIR. Past experiences have shown that pushing implementation ahead of finalized standards leads to confusion and inefficiencies. We recommend that ONC prioritize alignment with mature, stable standards and ensure that policy timelines reflect the current capabilities of the health IT ecosystem.

TD-11 a. Should this capability be revised to specify standardized API requirements for EHI export?

While the EHI Export capability has been in production since January 1, 2024, we believe it is time to revise the approach to ensure greater consistency and interoperability. The current flexibility granted to developers has led to variability in implementation. Moving forward, this capability should be standards-based and ideally implemented via API, with clearly defined FHIR resources for all relevant

data elements. This would promote uniformity, improve usability, and align with broader interoperability goals.

TD-11 b. Are there specific workflow aspects that could be improved?

A key workflow limitation stems from the lack of standardized formats in the original setup of the EHI Export capability. Without common standards, there is no assurance that one vendor's exported data can be successfully imported by another, which significantly hinders interoperability and usability across systems. Establishing consistent, standards-based formats would greatly enhance workflow efficiency and data portability.

TD-11 c. Should CMS consider policy changes to support this capability's use?

There is significant uncertainty about CMS's role in supporting the EHI Export capability, as this requirement originates from ONC policy, not CMS. Given that the current implementation lacks standardization, any policy mandating its use would be ineffective and potentially burdensome. The functionality, as it stands, is not practically usable and has diverted resources from more impactful, standards-based initiatives. Future efforts should focus on advancing API-based solutions using defined FHIR resources to ensure meaningful interoperability and real-world utility.

TD-12 Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

We question whether CMS is the appropriate entity to endorse non-CMS data sources and networks, as this responsibility more logically aligns with ONC and its existing certification framework. If such endorsements are pursued, they should be based on a clear, limited set of interoperability criteria and potentially managed as an extension of ONC's certification program. Additionally, any endorsement model must consider the value proposition and incentives for data sources to participate, as voluntary certification efforts in the past have seen limited uptake without clear benefits.

TD-13 a. What are the primary obstacles to this?

Expanding API access to the full scope of a patient's electronic health information (EHI) presents significant opportunities for innovation, care coordination, and patient empowerment. However, a primary obstacle is the lack of comprehensive FHIR resources to represent the full breadth of data stored in today's diverse and complex EHR systems. The variability in data scope across different types of health IT products—such as specialty-specific versus integrated care platforms—further complicates standardization and implementation.

TD-13 b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

The primary trade-off between USCDI and full EHI lies in the balance between standardization and data comprehensiveness. USCDI represents a defined, coded subset of data elements that support interoperability, while full EHI encompasses a broader, often unstandardized range of information. There is no simple mechanism to instantly standardize all EHI, and assuming otherwise reflects a

misunderstanding of the complexity involved. Robust, consensus-based standards take time to develop but are essential for scalable, meaningful data exchange. Policy efforts should remain grounded in the reality that interoperability depends on the maturity and adoption of such standards.

TD-14 a. How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?

As required by certification criteria, we support patient, provider, and system-level FHIR API use cases. Patient-facing endpoints are published and searchable by organizational identifiers, though these identifiers may vary in format and reliability. Provider and system endpoints are typically not published, as access is governed by contractual relationships and use-case-specific integrations. Our network includes clients across all specialties and geographic settings—urban, suburban, and rural. Any vendor that meets the standards and completes registration is eligible to connect, but unrestricted access to all endpoints is neither required nor practical.

TD-14 b. How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?

We support a range of connection methods—including FHIR G10 endpoints, TEFCA (for specific products), IHE XCA, and proprietary APIs—each serving different use cases. Over the years, we've supported hundreds of developers through proprietary APIs, which continue to play a vital role alongside FHIR due to unique EHR-specific functionalities that are not yet standardized. While FHIR is ideal for many interoperability scenarios, proprietary APIs remain essential for complex workflows such as order placement or high-frequency data reporting. The ecosystem requires a flexible, multi-tool approach, recognizing that no single standard can meet all needs today.

TD-15 a. How would increased use of bulk FHIR improve use cases and data flow?

While FHIR Bulk API offers significant potential—especially when enhanced with features like dynamic lists, subscription models, and CDS Hooks—its effectiveness depends heavily on the maturity of supporting infrastructure and standards. One challenge is that FHIR Bulk may not be the best fit for all workflows, particularly those requiring high-frequency data pulls or complex EHR-specific actions. Additionally, care must be taken to avoid indiscriminate data ingestion into EHRs, especially from patient-facing apps, which could overwhelm systems with irrelevant or unvalidated data. Ultimately, FHIR Bulk should be seen as one tool among many, best used when aligned with well-defined use cases and supported by scalable, standards-based networks.

TD-15 b. What are the potential disadvantages of their use?

While FHIR Bulk API offers significant potential—especially when enhanced with features like dynamic lists, subscription models, and CDS Hooks—its effectiveness depends heavily on the maturity of supporting infrastructure and standards. One challenge is that FHIR Bulk may not be the best fit for all workflows, particularly those requiring high-frequency data pulls or complex EHR-specific actions. Additionally, care must be taken to avoid indiscriminate data ingestion into EHRs, especially from

patient-facing apps, which could overwhelm systems with irrelevant or unvalidated data. Ultimately, FHIR Bulk should be seen as one tool among many, best used when aligned with well-defined use cases and supported by scalable, standards-based networks.

TD-17 Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

To ensure the viability and meaningful use of healthcare data sharing networks, CMS and ONC—alongside broader HHS leadership—should focus on enabling data exchange that serves clear, actionable purposes rather than promoting data flow for its own sake. Expanding participation from federal and state agencies, such as public health departments and the Social Security Administration, requires both funding and technical readiness. Additionally, existing integrations—especially those that address state-by-state variability—should not be dismantled in favor of duplicative certification requirements. Instead, efforts should prioritize building on proven infrastructure, incentivizing participation, and aligning resources to support scalable, outcome-driven data exchange.

TD-18 c. Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

A significant portion of healthcare actor categories defined under Section 3022(a)(1) of the Public Health Service Act—such as pharmacies, laboratories, dialysis centers, and long-term care facilities—are not required to use certified health IT, or in some cases, any electronic systems at all. As a result, these entities often lack both the technical infrastructure and regulatory incentives to comply with information sharing requirements. Without corresponding disincentives or support, their participation in interoperability efforts remains limited. Addressing this gap will require both policy alignment and investment in standards and infrastructure for these under-resourced sectors.

TD-19 a. What are the current shortcomings in content, format, delivery, and timeliness?

There are several key shortcomings in the current implementation of price transparency, particularly in terms of content format, delivery, and timeliness. There is consensus that price information should be available to providers at the point of care to support informed decision-making with patients. However, the complexity of healthcare pricing—especially for procedures involving multiple providers or settings—makes this challenging. Current regulations, such as the No Surprises Act, offer some protections, but they often fall short in helping patients truly understand their out-of-pocket costs. Additionally, while hospitals are required to publish pricing data, the format and variability make it difficult for patients and providers to interpret and use effectively. Overall, while the intent of price transparency is supported, its practical implementation remains burdensome for practices and confusing for patients.

TD-19 b. Which workflows would benefit most from functional price transparency?

Functional price transparency would be most impactful within ordering workflows—particularly for diagnostic testing, imaging, and lab procedures—where patients often face significant out-of-pocket costs. For transparency to be effective, pricing information must be available to providers at the point of care, enabling real-time discussions with patients. However, current systems often lack the integration needed to support this, especially when prior authorization is required. Additionally, reinforcing cost estimates during scheduling workflows would help patients make informed decisions and avoid unexpected charges. Overall, while these workflows stand to benefit the most, achieving seamless transparency will require substantial coordination across systems and stakeholders.

TD-19 d. What would further motivate solution development?

Further development of effective price transparency solutions will require clear and supportive policy direction. Current challenges—such as lack of real-time pricing at the point of care, administrative burdens on providers, and inconsistent payer decisions—create significant barriers. Providers are not incentivized to engage in complex prior authorization processes, which are time-consuming, unreimbursed, and costly to manage. Additionally, inconsistencies in coverage decisions and communication gaps between payers and providers undermine trust and usability. Addressing these systemic issues through policy reform and streamlined, standardized processes would significantly motivate and enable more robust solution development.

Value-based Care Organizations

VB-1 What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

To encourage broader adoption of digital health management and care navigation tools among APMs such as ACOs and MSSP participants, incentives should align with measurable improvements in patient engagement, outcomes, and cost savings. This could include enhanced shared savings opportunities, performance bonuses tied to digital engagement metrics, or streamlined integration support. However, significant barriers remain—chief among them is the overreliance on Epic as a one-size-fits-all solution for value-based care. Many organizations are hesitant to adopt best-of-breed digital tools due to (a) sunk costs in Epic, (b) fears of disrupting provider workflows, and (c) concerns from PSEC (Provider Systems Executive Committees) about governance, security, and interoperability. Addressing these barriers with targeted funding, technical support, and policy flexibility could unlock greater innovation and effectiveness in digital care delivery.

VB-3 a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.

Essential health IT capabilities for value-based care arrangements include electronic result interfacing, real-time ADT notifications, and point-of-care alerting to support timely and informed clinical decision-making. Additionally, meaningful clinical context through patient summaries and robust chronic care management workflow systems are critical to enabling coordinated, proactive, and patient-centered care.

VB-3 b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

In addition to foundational capabilities like electronic result interfacing, ADT notifications, and chronic care management workflows, point-of-care clinical documentation improvement (CDI) tools have proven especially valuable in value-based care arrangements. These tools enhance documentation accuracy and coding completeness in real time, supporting better quality measurement, risk adjustment, and care coordination.

VB-4 What are the essential data types needed for successful participation in value-based care arrangements?

Successful participation in value-based care arrangements requires access to a comprehensive set of data types, including patient demographics, clinical context from EHRs, lab and diagnostic results, ADT feeds, claims data, pharmacy information, and social determinants of health (SDOH). These data elements are critical for accurate patient attribution, risk stratification, care coordination, and performance measurement.

VB-6 What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

To better support APMs in value-based care, the ONC Health IT Certification Program should expand to include capabilities for capturing and utilizing social determinants of health (SDOH) data, which are critical for identifying barriers to patient adherence and access. Additionally, incorporating risk adjustment criteria at the point of care would enable more accurate, real-time decision-making and improve alignment with value-based performance metrics.

VB-11 What specific interoperability challenges have you encountered in implementing value-based care programs?

A key interoperability challenge in implementing value-based care programs is the significant variance in API capabilities across the long tail of electronic health record (EHR) systems. This inconsistency

hinders standardized data exchange, complicates integration efforts, and limits the scalability of digital tools essential for care coordination and performance measurement.

VB-15 How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

A nationwide provider directory of FHIR endpoints would significantly enhance access to patient data and improve clarity around claims data sources by enabling more efficient, standardized, and automated data exchange across healthcare entities. To maximize its effectiveness, the directory should include key data elements such as Tax Identification Number (TIN), National Provider Identifier (NPI), organization name, physical address, and active/inactive status of endpoints to ensure accurate routing and trust in data transactions.

This concludes Veradigm's comments on the Request for Information. We appreciate the opportunity to provide these detailed thoughts, suggestions and concerns and look forward to continued collaboration to improve patient care.

Sincerely,

Tina Joros, JD
Vice President – Policy & Innovation
Veradigm LLC